Group Therapy Progress Note Template

GIRP and DAP formats for adult IOP and PHP groups, with a filled-in example

One note per member, per group. The **session** is shared; the **note** is about one person: their goal, what was delivered, what they actually did or said, and what happens next.

The two formats

GIRP	**G**oal from this member's plan · **I**ntervention you delivered · **R**esponse you observed and what the member reported · **P**lan or next step
DAP	**D**ata: what was delivered and what you observed or the member reported · **A**ssessment: your clinical interpretation, tied to the goal · **P**lan or next step

Use the format your program requires. Both hold the same information in a different order.

Before you write

- **Check your program's required fields.** Date, service type, start and stop time, location, credentials and signature rules vary by program, state and payer.
- **Separate what you saw from what was said.** "Member described..." or "Member stated..." is different from "Member demonstrated...".
- **Record attendance as attendance.** Being present, watching or passing is not evidence of understanding or progress.
- **Leave other members out.** No names, initials or disclosures of other group members in this person's record.
- **Write a real plan.** A step the member chose, or "no step chosen", plus any follow-up and who owns it.

What's in this PDF

- Page 2: the blank Individual Encounter Note (Form 4 from Volume 1), with GIRP fields and a DAP alternative
- Page 3: the same fictional group written up in GIRP and in DAP

You may print this template for the groups you facilitate. Please don't edit it, remove its attribution or sell it. It is an educational example, not a complete note, billing guidance or legal advice, and it doesn't replace your program's documentation policy, supervision or clinical judgment.

Form 4: Individual Encounter Note - GIRP or DAP

One person and one actual encounter | Alternative formats; choose the format required locally.

Client ID / date / actual service, code, location, and start-stop time / clinician:

_____

_____

GIRP fields

G - Individual treatment goal and relevance of this encounter:

_____

_____

I - Intervention actually delivered, dose/time, accommodations, and format:

_____

_____

_____

R - Observed behavior / performance / declined or not assessed; then separately attributed member report:

_____

_____

_____

_____

P - Agreed next step or no step agreed; assistance, referral, owner, due time, acknowledgment:

_____

_____

_____

DAP alternative

D - Actual intervention, attendance/time, observed behavior, and attributed report:

_____

_____

_____

A - Interpretation linked to an individual goal, with the supporting observation; state limits:

_____

_____

P - Individual next step and any clinical action, owner, due time, handoff:

_____

_____

Do not infer improvement, safety, engagement, understanding, or readiness from attendance or a worksheet. Record urgent actions as they occur. This is not a completed note, validated assessment, or billing assurance.

Example: one skills group, two members

Adult IOP, 90-minute skills group, rolling admission. Session SK-CG-01: The Thought-Feeling-Behavior Connection and Common Thinking Traps. Names and details are invented.

Member A, returning member (GIRP)

G: Notice links between thoughts, feelings and actions when work stress rises (from member's plan).

I: Taught the thought-feeling-behavior chain and five common thinking traps; led a group chain on a made-up example, then optional personal practice with a moderate example. Worksheet offered with a no-writing option.

R: Member completed a chain aloud on a recent work email. Identified "mind reading" in the thought "My manager thinks I'm incompetent." Stated the chain was "easier to see written down." Asked how to use it when anxious.

P: Member chose to note one thinking trap per day on the worksheet. Facilitator to review at next skills group.

Member B, first day in group (DAP)

D: First group attended. Same session and interventions as above. Member received a brief newcomer orientation. Member watched the group example, did not complete the personal chain, and stated, "I'd rather listen today." Member asked one question about what "thinking trap" means; facilitator answered.

A: Engaged as an observer during first group; understanding of the skill not yet assessed. Question suggests interest in the core concept. No safety concerns observed or reported during group; no risk screening conducted in group.

P: No between-session step chosen. Offer the entry route of the same worksheet at the member's next skills group; primary clinician informed of first-group attendance per program routine.

Why these work

- Each note is about one person and names no one else.
- "Completed a chain aloud" is observed; "stated it was easier" is attributed to the member.
- Member B watching is recorded as watching, and the assessment says understanding wasn't assessed.
- Both plans say who does what next.

A note starter or documentation cue for all 82 sessions. Volume 1 of the Open-Enrollment Program Series gives GIRP language for every Skills, Psychoeducation and PHP session, plus documentation cues for Process groups. See thoughtkindpress.com/clinicians.html